

1. Administrative & Study Identification

Full Study Title: A Study to Compare the Efficacy of Nivolumab and Relatlimab Plus Chemotherapy vs Pembrolizumab Plus Chemotherapy for Stage IV/Recurrent Non-squamous Non-small Cell Lung Cancer With PD-L1 Expression $\geq 1\%$ **Short Title/Acronym:** RELATIVITY-1093 **Protocol Number:** CA224-1093 **NCT Number:** NCT06561386 **Posting ID:** 795506410900607235 **Sponsor Name:** Bristol Myers Squibb **Investigational Product:** Nivolumab + Relatlimab (Fixed-Dose Combination) vs. Pembrolizumab **Phase of Development:** Phase III **Trial Status:** Recruiting **Medical Monitor:** Bristol Myers Squibb Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective

- Primary Objective:** To compare the efficacy of Nivolumab and Relatlimab in combination with chemotherapy to Pembrolizumab with chemotherapy in participants with stage IV or recurrent non-squamous Non-small Cell Lung Cancer (NSCLC) with PD-L1 expression $\geq 1\%$.
- Secondary Objectives:** To evaluate Overall Survival (OS) in participants with PD-L1 $\geq 1\%$, Progression-Free Survival (PFS) per RECIST v1.1 per blinded independent central review (BICR), and Overall Response Rate (ORR).

2.2 Background & Rationale To address the need for improved first-line treatments in advanced NSCLC. While pembrolizumab plus chemotherapy is a current standard-of-care, targeting LAG-3 in combination with PD-1 inhibition has shown significant promise. Findings from the phase 2 RELATIVITY-104 study demonstrated that adding the LAG-3 inhibitor relatlimab to nivolumab plus chemotherapy provided a clinical benefit versus nivolumab plus chemotherapy alone in this patient population. RELATIVITY-1093 aims to confirm if this synergistic regimen can propel efficacy beyond standard anti-PD-1 therapy.

3. Study Design & Methodology

3.1 Design Framework

- Study Type:** Interventional
- Control Type:** Active Comparator (Pembrolizumab + Chemotherapy)
- Blinding:** Open-label

- **Randomization:** Randomized

3.2 Planned Enrollment & Duration

- **Target N:** 1000 participants
 - **Number of Sites:** Multicenter global clinical trial (Over 118 locations)
 - **Estimated Study Start:** 07-Oct-2024
 - **Estimated Primary Completion:** 30-Jul-2030
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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Males and females ≥ 18 years of age. 2. Histologically confirmed Stage IV or recurrent Non-small Cell Lung Cancer (NSCLC) of non-squamous histology. 3. No prior systemic anti-cancer therapy given as primary therapy for advanced or metastatic disease. 4. Measurable PD-L1 $\geq 1\%$ Tumor Cell (TC) score by the investigational VENTANA PD-L1 (SP263) CDx Assay conducted by a central laboratory. 5. Measurable disease by CT or MRI per RECIST v1.1 and ECOG performance status of ≤ 1 at screening. 6. Life expectancy of at least 3 months at the time of randomization.

4.2 Exclusion Criteria 1. Pregnant and/or breastfeeding. 2. Known EGFR, ALK, or ROS-1 mutations sensitive to available targeted inhibitor therapy (participants with unknown EGFR, ALK, or ROS-1 status are excluded). 3. Known BRAFV600E mutations sensitive to targeted therapy, activating RET mutations, or NTRK fusion gene alterations (participants with unknown or indeterminate BRAF, RET, or NTRK are eligible). 4. Untreated central nervous system (CNS) metastases or leptomeningeal metastases (carcinomatous meningitis). 5. Concurrent malignancy requiring treatment. 6. Active autoimmune disease, history of myocarditis, or history of interstitial lung disease/pneumonitis that required oral or IV glucocorticoids to assist with management. 7. Prior treatment with an anti-PD-1, anti-PD-L1, anti-PD-L2, anti-CTLA-4 antibody, or other antibody or drug targeting T-cell co-stimulation or checkpoint pathways.

5. Intervention & Treatment Plan

5.1 Investigational Regimen

- **Dosage/Frequency:** Standard cyclic oncology infusion schedules per protocol for checkpoint inhibitors and platinum-doublet chemotherapy (including Cisplatin [Trade name: Platinol, ATC Code: L01XA01]).
- **Route of Administration:** Intravenous (IV) Infusion.
- **Duration of Treatment:** Until disease progression, unacceptable toxicity, or study completion.

5.2 Concomitant & Prohibited Therapy

- **Permitted:** Standard antiemetics and supportive care for chemotherapy-induced side effects.
 - **Prohibited:** Concurrent systemic anti-cancer therapies, live vaccines, and systemic immunosuppressive treatments (e.g., high-dose corticosteroids) not indicated for AE management.
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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Scans, PD-L1 IHC testing, Mutation screening
Baseline	Day 0	Randomization, First IV Infusion, Safety Baseline	Interim
Interim	Per Cycle	Intravenous Treatment, Safety Labs, AE Monitoring, Tumor Imaging (RECIST 1.1)	End of Study
Follow-up	Final Scans, Overall Survival Tracking, Safety Follow-up		

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Overall Survival (OS) in randomized participants with PD-L1 1% to 49%. **Measurement Method:** Defined as the time from randomization to death due to any cause.

7.2 Secondary & Exploratory Endpoints * Overall survival (OS) in randomized participants with PD-L1 $\geq$ 1%. * Progression-Free Survival (PFS) as assessed by Response Evaluation Criteria in Solid Tumors version 1.1 (RECIST v1.1) per blinded independent central review (BICR). * Overall Response Rate (ORR). * Duration of Response (DOR) per RECIST 1.1 by BICR and Investigator.

8. Safety & Adverse Event Reporting

- **Adverse Event (AE) Monitoring:** Standard collection via regular onsite visits, lab monitoring, and physical exams during treatment cycles.
 - **Serious Adverse Events (SAEs):** Monitored continuously; immune-mediated adverse events (imAEs) closely tracked due to the immunotherapy regimen.
 - **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee to oversee safety signals and trial efficacy.
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9. Statistical Analysis Plan (SAP) Summary

- **Analysis Populations:** Intent-to-Treat (ITT) for primary efficacy (OS); Safety Set for all participants receiving at least one dose of the investigational product.

- **Sample Size Justification:** Target enrollment of 1000 participants, powered to detect a statistically significant improvement in Overall Survival for the dual checkpoint blockade (LAG-3 + PD-1) compared to the standard of care.
 - **Handling of Missing Data:** Standard oncology imputation methods; censoring for PFS/OS at the last known alive/progression-free date.
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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular onsite and remote monitoring of eCRFs, safety data, and protocol adherence. **Audit Trail:** Centralized imaging read (BICR) for unbiased RECIST 1.1 assessment; central lab processing for PD-L1 assays.

11. Study Identifiers & Governance

Primary ID: CA224-1093 **Registry Number:** NCT06561386 **Posting ID:** 795506410900607235 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** A Study to Compare the Efficacy of Nivolumab and Relatlimab Plus Chemotherapy vs Pembrolizumab Plus Chemotherapy for Stage IV/Recurrent Non-squamous Non-small Cell Lung Cancer With PD-L1 Expression $\geq 1\%$ **Official Regulatory Title:** A Phase 3, Randomized, Open-label Study of Nivolumab + Relatlimab Fixed-dose Combination With Chemotherapy Versus Pembrolizumab With Chemotherapy as First-line Treatment for Participants With Non-squamous (NSQ), Stage IV or Recurrent Non-small Cell Lung Cancer and With Tumor Cell PD-L1 Expression $\geq 1\%$

12. Clinical Framework (NSCLC Specific)

Primary Condition: Non-small Cell Lung Cancer (Metastatic non-small cell lung cancer) **Diagnosis Threshold:** Stage IV or Recurrent, PD-L1 expression $\geq 1\%$, no targetable actionable mutations **Medical Methodology:** Dual Immune Checkpoint Blockade (Anti-PD-1 + Anti-LAG-3) plus Platinum-Doublet Chemotherapy **Optimization Target:** Improved Overall Survival and Progression-Free Survival in first-line setting

13. Study Procedures & Methodology

Management Pathway: Standard first-line oncology clinical pathway **Education Protocol (HCP):** Investigator training on RECIST 1.1, BICR submission, and imAE management **Education Protocol (Patient):** Informed consent and infusion scheduling education

Quality Audit Mechanism: Central laboratory PD-L1 (SP263) testing & Blinded Independent Central Review (BICR) evaluation for scans

14. Observation & Follow-Up Schedule

Total Duration: Estimated Study Completion in August 2032 **Onsite**

Visit Cadence: Aligned with standard IV infusion chemotherapy/immunotherapy schedules (e.g., every 3-4 weeks) **Remote Monitoring**

Frequency: Intermittent survival follow-up post-progression

Checklist Review Interval: Prior to every treatment cycle

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
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Principal Investigator	[Name]	____	[DD-MMM-YYYY]	
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Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]	Lead
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Statistician | [Name] | ____ | [DD-MMM-YYYY] |